# Statement of Deficiency Report

Department of Health
P.O. Box 47874, Olympia, WA 98504-7874
TEL: 360-236-4732

Comprehensive Life Resources - Pearl Youth Residence
1201 South Proctor Street
Tacoma, WA 98405-2047
Agency Name and Address

Melissa Olson
Administrator

| Investigation | 09/18/20-10/20/20 | JAMC03 |
|---|---|---|
| Inspection Type | Investigation Dates | Investigator Number |
| 2020-13310 | RTF.FS.6107805 | Mental Health Services |
| Case Number | License Number | BHA/RTF Agency Services Type |

Please note that the deficiencies/violations/observations noted in this report are not all-inclusive, but rather were deficiencies/violations/observations that were observed or discovered during the investigation.

| Deficiency Number and Rule Reference | Observation Findings | Plan of Correction |
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| WAC 246-337-065(5)(d) Safety and security. The licensee must protect resident safety and security by developing written policies and procedures that are consistent with the requirements of this chapter and address: (5) Reporting to the department and other appropriate agencies, by the end of the next business day of the incident occurring, serious or undesirable outcomes that occur in the facility including: (d) Injuries resulting in an inpatient hospital stay. | Based on record review, interview, and policy and procedure review, the facility failed to develop and implement written policies and procedures that require reporting to the department an incident involving serious or undesirable outcomes, including injuries resulting in an inpatient hospital stay, by the next business day of the incident occurring for 1 of 1 residents reviewed (Resident#1).<br><br>Failure to develop and implement written policies and procedures that require reporting to the department an incident involving serious or undesirable outcomes, including injuries resulting in an inpatient hospital stay, by the next business day of the incident occurring may | |

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| | result in a lack of facility oversight, which may result in resident harm.<br><br>Findings included:<br><br>1. Record review of facility policy titled "Critical Incidents", dated 10/29/20, showed that it is facility policy to report within one working day to "regulatory and contractual organizations" incidents such as alleged or actual abuse or neglect of a resident, credible or actual harm to client or staff, medication errors, or suicide attempts. The document showed that it is policy to report immediately incidents such as unauthorized leave of a client under court order for involuntary treatment, and death or serious injury. The facility states, "Professional judgment shall be used to determine whether any incident falling outside the scope of this policy requires and receives notification to regulatory and contractor organizations". The policy did not show that it is facility policy to report serious or undesirable outcomes that occur in the facility, including injuries resulting in an inpatient hospital stay.<br><br>2. During an interview on 10/02/20 at 10:15 AM, Staff A, Administrator, stated that the facility had not sent an incident report to Department of Health (DOH) for any of the incidents involving Resident #1 eating non-food items because it did not meet the criteria for reporting.<br><br>3. Review of Resident #1's clinical record showed that Resident #1 was involved in an incident that had serious or undesired outcomes and was admitted to | |

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| | the hospital for injuries that resulted in an overnight stay, based on the following:<br><br>a. Review of a facility incident report, dated 09/15/20, showed that Resident #1 was transferred to the hospital on 09/15/20 after Resident #1 "refused to change [their] tampon, stating that [they] wanted to leave it in so that [they] get toxic shock syndrome". Staff B, Medical Director, ordered that Resident #1 be sent to the ER. The document showed that while at the ER, facility staff received an update from hospital staff. Resident #1 reported to hospital staff that they had eaten 3 tampons [while at the facility] and was "taken to the OR [operating room] for scope for removal".<br><br>b. Review of a facility incident report, dated 09/15/20, showed that "[Resident #1] left the building in the suicide smock at 11:15 via EMS to [emergency room]". Review of Resident #1 observation record, dated 09/16/20, showed that Resident #1 returned from the hospital to the facility around 7:00 PM on 09/16/20.<br><br>c. Review of a document titled "Staff Pass Ons" 9/12/20 – 09/22/20, showed that on 09/16/20 staff discussed Resident #1s current medical status and staff contacted the hospital to check on the status of Resident #1. "ER update. GI endoscopy clear. No distention. Has been eating, drinking normally. Stools normal... She will return (to the facility) later tonight [from the hospital]..."<br><br>d. During an interview on 10/01/20 at 2:00 PM, Individual E, Social Worker at the hospital where the facility sent Resident #1 to on 09/15/20, stated that | |

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| | while in the hospital, Resident #1 underwent procedures, including "scoping" (a procedure in which a small camera in inserted into the stomach), which is considered dangerous. | |

**Introduction**

We require that you submit a plan of correction for each deficiency listed on the statement of deficiency form. Your plan of correction must be Submitted to DOH within fourteen calendar days of receipt of the list of deficiencies.

You are required to respond to the statement of deficiencies by submitting a plan of correction (POC). Be sure to refer to the deficiency number. If you include exhibits, identify them and refer to them as such in your POC.

**Descriptive Content**

Your plan of correction must provide a step-by-step description of the methods to correct each deficient practice to prevent recurrence and provide information that ensures the intent of the regulation is met.

An acceptable plan of correction must contain the following elements:

- The plan of correcting the specific deficiency;
- The procedure for implementing the acceptable plan of correction for the specific deficiency cited;
- The monitoring procedure to ensure that the plan of correction is effective and that specific deficiency cited remains corrected and/or in compliance with the regulatory requirements;
- The title of the person responsible for implementing the acceptable plan of correction.

Simply stating that a deficiency has been "corrected" is not acceptable. If a deficiency has already been corrected, the plan of correction must include the following:

- How the deficiency was corrected,
- The completion date (date the correction was accomplished),
- How the plan of correction will prevent possible recurrence of the deficiency.

**Completion Dates**

The POC must include a completion date that is realistic and coinciding with the amount of time your facility will need to correct the deficiency. Direct care issues must be corrected immediately and monitored appropriately. Some deficiencies may require a staged plan to accomplish total correction. Deficiencies that require bids, remodeling, replacement of equipment, etc., may need more time to accomplish correction; the target completion date, however, should be within a reasonable and mutually agreeable time-frame.

**Continued Monitoring**

Washington State Department of Health
PLAN OF CORRECTION

| Investigation Dates: 9/18/20-10/20/20<br>Investigator Number: JAMC03<br>License Number: RTF.FS.6107805<br>Case Number: 2020-13310 | Comprehensive Life Resources<br>Pearl Youth Residence (PYR)<br>1201 South Proctor Street<br>Tacoma, WA 98405-2047 |
|---|---|

| Deficiency Number and Rule Reference | Correction | Responsible Party | Implementation Date | Maintaining Correction |
|---|---|---|---|---|
| WAC 246-337-065(5)(d) Safety and security. The licensee must protect resident safety and security by developing written policies and procedures that are consistent with the requirements of this chapter and address: (5) Reporting to the department and other appropriate agencies, by the end of the next business day of the incident occurring, serious or undesirable outcomes that occur in the facility including: (d) Injuries resulting in an inpatient hospital stay. | 1. Unit Supervisor will check that all Incident Reports are completed before leaving their shift. If they cannot complete the BIR, they will let an alternate staff know. | 1. Unit Supervisor | 1.11/13/20 | Administrative Services Manager will double check the Incident Reports are complete and track them (see Correction 3) |
| | 2. New process: when staff write an Incident Report, they will alert the Administrative Services Manager | 2.Program Director will train staff when to alert the administrative Services Manager | 2.11/20/20 | 2.Administrative Services Manager will track the reports (see Correction 3) |
| | 3. Develop a new policy with incident reporting procedures specific to PYR. The new policy will refer to the reporting grid. Refer to WACs. | 3. Program Director | 3. 2/1/21 | 3.Program Director will review the new policy when complete |
| | 4. Update reporting grid. | 4. Administrative Services Manager | 4. 3/1/21 | 4. Administrative Services Manager will review reporting grid with Program Director and CLR Quality Improvement Associate by 3/1/21 |

STATE OF WASHINGTON

# DEPARTMENT OF HEALTH

November 24, 2020

Comprehensive Life Resources – Pearl Youth Residence
1201 South Proctor Street
Tacoma, WA 98405-2047

Re: Case Number: 2020-13309 and 2020-13310
License Number: BHA.FS.61067737 and RTF.FS.61067805
Acceptable Plan of Correction
Date(s) of Investigation: 09/18/20 – 10/20/20

Dear Melissa Olson:

This letter is to inform you that after careful review of the Plan of Correction (POC) you submitted for the investigation recently conducted at your facility, the Department has determined that the POC is acceptable. You stated in your plan that you will implement corrective actions by the specified timeline. By this, the Department is accepting your Plan of Correction as your confirmation of compliance.

Based on the scope and severity of the deficiencies listed in your statement of deficiency report, the Department will not conduct an unannounced follow-up compliance visit to verify that all deficiencies have been corrected.

The Department reserves the right to pursue enforcement action for any repeat and/or uncorrected deficiencies based on applicable statute and rules.

Investigator: JAMC03
Department of Health
HSQA/Office of Health Systems Oversight
PO Box 47874
Olympia, Washington 98504-7874